

# INFORMED CONSENT FOR HAEMODIALYSIS

|               |          |       |         |
|---------------|----------|-------|---------|
| Patient Name: |          | Age : | Gender: |
| UHID No.      | OP/IP no |       | Date:   |

## Diagnosis:

I have been explained the condition of the patient (Acute / Chronic Renal Failure) by the Nephrologist in a language understood by me. I am aware that Dialysis is a treatment procedure for kidney failure wherein blood is filtered through a dialysis membrane and reintroduced into the body. Per se, it is not a curative treatment.

Details of procedure: Haemodialysis (Intermittent), CRRT and SLED are similar procedures and use the same principle (membrane filtration). At the start of the session, needles are inserted into the AV fistula/graft and taped into place. In some patients, an existing vascular access port (e.g. Internal Jugular/Subclavian vein) may be used to connect the dialysis machine instead of an AV fistula/graft. Blood is slowly removed and transferred through a tube to a dialysis machine which is made up of a series of membranes. These membranes filter waste products from blood, which are passed into the dialysate fluid. The 'dirty' dialysate fluid is pumped out of the dialyser and the 'clean' blood is passed back into the body through the second needle. The needles are removed at the end of the session. A dialysis session takes around four hours to complete because blood needs to be removed, filtered and replaced slowly. In CAPD, tube is inserted into the abdominal cavity and the peritoneum is used as the filtering membrane.

## Full Name of the procedure:

|                                                                           |
|---------------------------------------------------------------------------|
| <input type="checkbox"/> INTERMITTENT HAEMODIALYSIS (IHD)                 |
| <input type="checkbox"/> CONTINUOUS RENAL REPLACEMENT THERAPY (CRRT)      |
| <input type="checkbox"/> SLOW LOW EFFICIENCY DIALYSIS (SLEDD)             |
| <input type="checkbox"/> CONTINUOUS AMBULATORY PERITONEAL DIALYSIS (CAPD) |

I understand the process of haemodialysis that is done and monitored by well experienced dialysis technicians. The procedures involved in haemodialysis have been explained to me by the doctors and dialysis technicians. I consent to the patient and allow the doctors to the dialysis for the patient as and when required according to the clinical condition.

**Benefits:** Provide an alternative for reduced kidney function , Removal of waste products from the blood, Removal of extra fluid from the body, Improving quality of life

## Possible risks and complications:

- Fluctuation in blood pressure (low BP i.e. Hypotension or high BP i.e. Hypertension) is a well-known side effect of Haemodialysis and may require discontinuation of the dialysis session. This can be corrected by placing the patient in a head-low position and adjusting the fluid intake.
- Some patients may develop muscle cramps especially towards the end of the procedure. These can be overcome by massaging but may sometimes require treatment by fluids or medicines.
- Irregularity of heart rhythm may occur in some patients and may require investigation (like blood tests) and treatment by medicines.
- Some patients may develop fever (febrile reaction). This may be severe enough to require discontinuation of dialysis and treatment by medicines.

- Development of symptoms like nausea, vomiting, restlessness and headache. These are usually self-limiting but may require modification of the dialysis procedure and treatment with medicines.
- Some patients may develop an allergic reaction to the dialyser membrane, IV fluid, drugs and/or disinfectants. This may require discontinuation of the session and treatment by medicines.
- Stroke, chest pain (angina), cardiac arrest and air embolism (presence of air bubble in the blood circulation) are uncommon complications of Haemodialysis, and may require hospitalization and/or admission to an Intensive Care Unit.
- Some patients can develop internal clotting of blood in the dialysis circuit. Blood thinner (like Heparin) is administered to overcome this.
- Some patients may develop breakdown of red blood cells (haemolysis). This may rarely be life threatening and require discontinuation of dialysis and treatment of symptoms.
- Possibility of contracting infections like Hepatitis B, Hepatitis C, HIV etc. Risk of infection of abdominal cavity (peritonitis) is higher in patients on CAPD.

**Possible alternatives:** Kidney transplant, Peritoneal dialysis

- i. Though all the precautionary measures will be taken by the dialysis staff some of these complications are inevitable. I also consent to do any procedure and to administer any drug during this complication. I also understand the need of dialysis for the patient, which substitutes the function of the kidney. I also entirely agree and oblige to the rules and regulation of the Dialysis Department and the Hospital (dialysis schedule and timings). I understand that For patients who are Dialysis dependent, fresh consent will be obtained prior to start of first session every month. This is valid for each dialysis procedure conducted during the calendar month provided the endorsement is done each time in the table provided on last page of consent form. For patients (not on Maintenance Dialysis) undergoing acute Dialysis, a fresh consent will be taken for each dialysis. Sometimes dialysis may be delayed due to technical problems of the machines or in case of emergencies. It has been explained to me the risk involved on not following the doctor's order (interdialytic weight gain and drugs). I agree to the Nephrologist's decision regarding the dialysis prescription for the patient.

|                                          | Name                                       | Signature | Date | Time |
|------------------------------------------|--------------------------------------------|-----------|------|------|
| Patient                                  |                                            |           |      |      |
| Patient representative with relationship |                                            |           |      |      |
| Reason for surrogate consent             | Patient is unable to give consent because: |           |      |      |
| Doctor                                   |                                            |           |      |      |
| Witness                                  |                                            |           |      |      |
| Interpreter (if necessary)               |                                            |           |      |      |